

SUBJECTIVE:

- Chief Complaint: Sore throat and cough.
- History of Present Illness: The patient reports an overall illness duration of just over 5 days, with an acute onset of a constant, scratchy sore throat today, rated 5/10 in severity and localized to the throat without radiation. No specific alleviating or aggravating factors were identified. A dry cough started yesterday, occurring in episodes lasting approximately 4 hours each, with no associated chest pain or wheezing. The patient also notes a constant sour taste in the mouth, present only during this current illness. No previous similar episodes or treatments were reported.

OBJECTIVE:

- Expected physical exam findings:
- Oropharynx: Expected findings include mild to moderate erythema of the posterior pharynx, possibly with some cobblestoning, but no tonsillar exudates, significant hypertrophy, or peritonsillar swelling.
- Neck: No significant cervical lymphadenopathy or tenderness on palpation. Thyroid non-tender.
- Lungs: Clear to auscultation bilaterally; no crackles, rhonchi, or wheezing expected.
- Vital signs: Not available for review.

ASSESSMENT:

- Primary Diagnosis: Acute Viral Pharyngitis / Upper Respiratory Infection (URI)
- The patient's presentation with a scratchy sore throat, dry cough, and a sour taste during an acute illness, without reported fever or other red flags, is highly consistent with a common viral infection. The sour taste may be secondary to post-nasal drip or mild reflux.
- Differential Diagnoses:
- Streptococcal Pharyngitis: Less likely given the scratchy sore throat and absence of reported fever, exudates, or tender lymphadenopathy.
- Allergic Rhinitis/Pharyngitis: Possible, but the acute illness context favors an infectious etiology.
- Gastroesophageal Reflux Disease (GERD): Can cause a sour taste and throat irritation, but likely secondary or exacerbated in this acute context.

PLAN:

- Tests: No specific diagnostic tests are indicated given the strong clinical suspicion for a viral etiology.
- Medications:
- Over-the-counter pain relief (e.g., Acetaminophen or Ibuprofen) for throat pain, as needed.
- Throat lozenges or sprays for symptomatic relief.
- Over-the-counter cough suppressant (e.g., Dextromethorphan) for dry cough, as needed.
- Patient Advice:
- Encourage adequate hydration and rest.
- Advise avoiding irritants like smoke and vaping.
- Follow-up Instructions: Return to clinic or seek urgent care if symptoms worsen significantly (e.g., high fever, difficulty breathing, severe difficulty swallowing), or if there is no improvement within 7 to 10 days.